

LENSKART ENGAGEMENT BREAKDOWN

PM Case Study · Problem Diagnosis & System Framing

Lenskart completes the job-to-be-done so effectively that users have no reason to return.

The core failure mode is **completion, not dissatisfaction**. Once eyewear is delivered, users mentally close the loop, leading to a sharp collapse in engagement despite high satisfaction with the purchase experience.

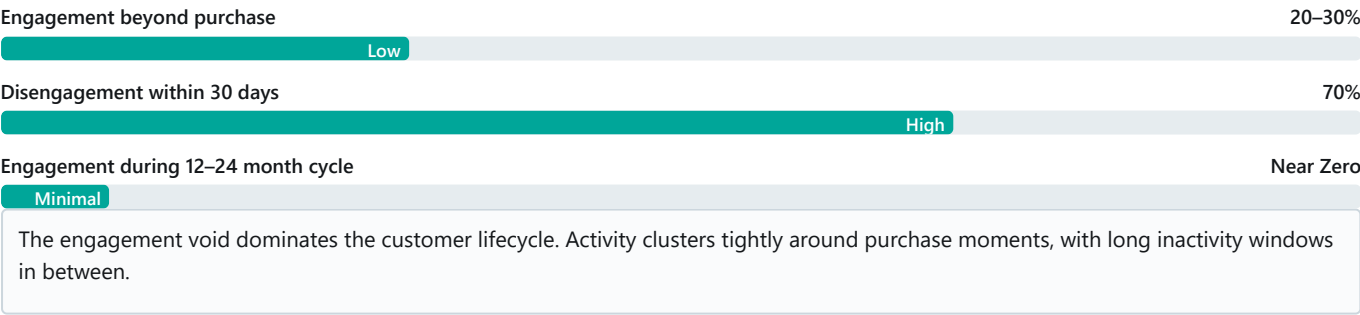
What Happens After Purchase

Stage	User Mental Model	Engagement Outcome
Need identified	"I can't see clearly"	High intent, active engagement
Purchase completed	"I bought glasses"	Transactional usage only
Delivery received	"My problem is solved"	Cognitive closure
Post-delivery	"No reason to return"	Engagement collapse

What This Looks Like at Scale



Engagement Decay Across the Lifecycle



Why This Is a Structural Risk

Mindshare Loss

- Extended inactive periods post-purchase
- Offline opticians fill interim care needs
- Lenskart loses top-of-mind presence

Capped Lifetime Value

- Economics driven by replacements
- No compounding engagement behavior
- Revenue ceiling hard-coded into lifecycle

Dormant First-Party Data

- Prescriptions and eye history captured once
- Store and family signals underutilized
- No engagement layer to activate data

Commerce works. Care doesn't.
This is not a transaction problem — it is an engagement architecture problem.

WHY EYE CARE IS EPISODIC IN INDIA

From our market research we understood that eye care in India is constrained by access and infrastructure, not by lack of awareness. Episodic behavior is structurally enforced at the category level.

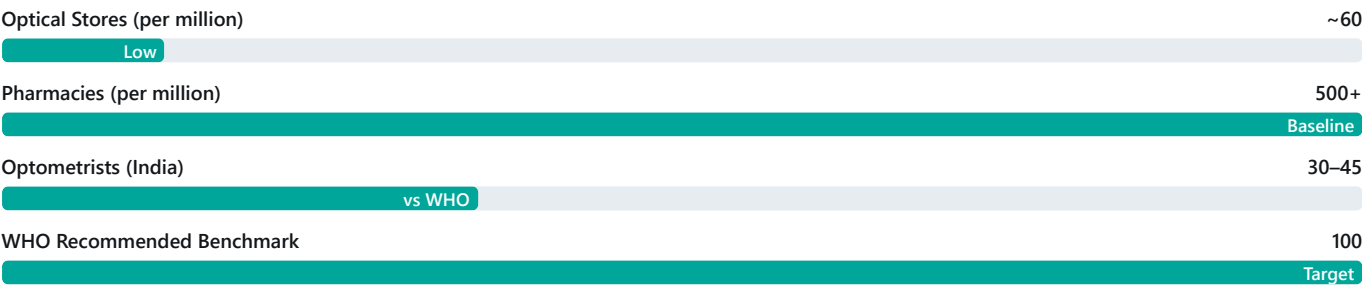
Research Methodology

- 30+ in-depth user interviews (Tier 1–3)
- Secondary research: WHO, Indian health surveys, industry reports
- Review mining across eyewear & health platforms

Market Reality: Eye Care Access in India



Infrastructure Density Comparison



Market Signals & Structural Implications

Market Data Signals

- **23%** organized optical retail penetration in India
- **77%** eye care handled by unorganized local opticians
- **Metro vs Tier-2+ gap:** 53% vs ~32% organized access
- **Lenskart scale:** 2,723 stores across 431 cities
- Majority of urban digital workers report chronic eye strain
- Strong preference for blue-light blocking lenses
- **₹2.5–3.8 trillion** annual economic loss from uncorrected refractive errors

What This Structurally Creates

- **Access scarcity → reactive behavior:** Users visit opticians only when vision deterioration becomes disruptive, not for routine monitoring.
- **Unorganized care blocks continuity:** Local opticians optimize for episodic sales, not long-term eye health tracking.
- **Urban strain without care loops:** Screen-induced discomfort is common, but lacks system-led follow-ups or cadence.
- **Awareness without urgency:** Vision decline is gradual and cognitively compensated for, delaying action until crisis.
- **Category-level outcome:** Eye care collapses into replacement cycles (18–24 months), not an ongoing health relationship.

So What?
Episodic care behavior structurally enforces **low non-transactional MAU**. Without

AWARENESS EXISTS. ACTION DOESN'T.

High awareness + high stated preference + massive economic cost still results in **zero sustained preventive eye-care habits**.

Key Research Questions

- Why does engagement drop immediately after eyewear delivery?
- How do users think about eye care between purchases?
- What actually triggers eye-care behavior — discomfort, reminders, or habit?
- What prevents users from acting on eye-care knowledge?
- Who is the real decision-maker — wearer, purchaser, or family manager?
- How many people does one user typically manage eye care for?
- Which channels do users respond to for non-transactional engagement?

USER REALITY

"I know I should get my eyes checked, but unless something feels wrong, I don't think about it."

"My glasses work fine, so there's no reason to open the app again."

"Eye care isn't like fitness — there's nothing to do daily."

Source: 30+ interviews across Tier 1–3 cities (Full data: internal research spreadsheet)

BEHAVIORAL EXPLANATION

Using the Fogg Behavioral Model:

Motivation
Moderate

Ability
Low
(effort, access)

Trigger
Missing

Awareness alone fails when ability is low and triggers are absent

The Awareness–Action Chasm

User Belief	Observed Reality
"Eye checks are important"	No perceived harm in delaying
"I'll go if there's a problem"	Vision decline is gradual and compensated for
"I should take care of my eyes"	No system reminds or guides next action
"I just bought new glasses"	Problem mentally classified as solved

Why Education Alone Fails:
Users already know eye care matters. Without low-friction ability and system-driven triggers, awareness does not convert into repeat action or engagement.

User Pain Points Identified

- No urgency without visible decline or discomfort
- No default reminder, follow-up, or care cadence
- No perceived benefit between purchases
- Eye care mentally classified as "done" after buying glasses
- Care actions feel optional, effortful, and deferrable

Review Insight:
This is not an education problem.

THE FAMILY REALITY LENSKART IGNORES

Research Insight #2 · Family Decision Structure
Based on 30+ user interviews

In Indian households, the person who buys eyewear is often not the person who uses it — and rarely the only one involved in the decision.

PRIMARY PERSONA: FAMILY CARE COORDINATOR



Renu Singh (Representative)

- Age: 38
- City: Tier-2 (Indore / Lucknow)
- Household Income: ₹6–8 LPA
- Digital Literacy: Comfortable with apps, not proactive
- Manages eye care for: self + parents + 1 child

Eye care decisions are centralized around one digitally capable family member, not individual end-users.

OBSERVED PATTERN

62%
Manage Eye Care for Family

38%
Manage Only Self

Average family size managed: **3–4 members**

Family Decision Flow

Parent ← **Renu (Coordinator)** →
Child
Grandparent ↑

Household-Level Reality

2.5–3.5

AVG. MEMBERS PER HOUSEHOLD

1

PRIMARY BUYER PER FAMILY

Multiple

DEPENDENTS MANAGED

Evidence-Backed User Personas

Persona	Who They Are	Role in Eye Care	Primary Pain Point
Renu Singh (Family Healthcare Manager)	38, working parent, manages 6 family members	Purchases eyewear, schedules visits, monitors children & parents	Fragmented tracking, no reminders, no single family dashboard
Abhishek (Child / Elder)	Dependent family member	Passive recipient of care Rarely initiates follow-ups	Fully dependent on others to notice and act

Persona-Level Pain Point

- Individual-only design ignores family coordination reality
- Reminders reach wearers, not decision-makers
- No shared visibility into check-ups, prescriptions, or due dates
- Care continuity depends on memory, guilt, and chance discovery

Research Insight:
Lenskart assumes an individual user, but eye care is managed by a **household coordinator**.

FASHION INTEREST EXISTS. BUT OFF PLATFORM.

Users care deeply about how they look in glasses — but **that interest rarely translates into opening Lenskart between purchases.**

Key Research Questions

- Where do users browse eyewear styles between purchases?
- What triggers fashion browsing if not buying intent?
- Why doesn't Lenskart become the default discovery destination?
- What role does fashion play outside replacement moments?

Observed Fashion Behavior

User Segment	Fashion Interest Level	Where Browsing Happens	Why Lenskart Is Not Used
Rohan Kumar (Urban Professional)	High	Instagram, Pinterest, YouTube	No reason to browse without buying intent
College Students	Very High	Instagram, K-pop / influencer content	Seen as a store, not self-expression platform
Working Parents (40+)	Moderate but emotional	Google, Pinterest, offline references	Fashion framed clinically, not contextually
Tier-2 Pragmatists (Abhinash Joshi)	Low–Moderate	Offline opticians, WhatsApp images	Fashion tied only to replacement or price arbitrage

User Pain Points Identified

- Fashion browsing feels disconnected from eye care
- No lightweight way to “just look” on Lenskart
- Browsing implies buying — which users aren't ready for
- No social or contextual relevance outside purchase moments

Research Insight:
Fashion interest is **ambient and continuous**, but Lenskart frames fashion as a **transactional step**.
Without a non-purchase browsing mode, fashion demand leaks to external platforms.

CATEGORY LEVEL CONSTRAINTS SHAPING EYE-CARE ENGAGEMENT

Lenskart’s engagement challenges are not product failures — they emerge from **structural constraints of the eye-care category** that no player in India has fully solved.

Direct & Adjacent Competitors

Player	What They Own	Where They Fail
Local Opticians	Trust, family memory, post-purchase care	No scale, no data, no systematization
Practo / Health Apps	Medical credibility, family records	Episodic usage; eyewear not core
Fashion Platforms	Daily browsing, inspiration	No care, no prescriptions, no health trust
Lenskart (Today)	Transaction efficiency, scale	No continuity between care, family, and fashion

What Reviews Reveal

- “Useful for ordering, nothing after delivery” → **zero post-purchase utility**
- Adjustments and comfort handled by local opticians → **care outsourced offline**
- Notifications remembered as discounts → **early trust erosion**
- No mention of eye-care routines, reminders, or family tracking → **no habit formation**

Research Insights Summary

- Eye care is structurally reactive, not habit-forming
- Awareness exists, but action lacks triggers and cadence
- Family-managed care breaks individual engagement models
- Fashion interest leaks to external platforms
- Digital tools lack trust, continuity, and visible payoff

The 8 Forces Shaping Eye-Care Engagement

Constraint	What It Means in Practice	Impact on Engagement
Infrastructure Scarcity	Low optician / optometrist density	Care is effortful and infrequent
Preventive Habit Vacuum	No culturally reinforced eye-care routine	No default cadence to anchor engagement
Invisible Decline	Vision worsens gradually without signals	No urgency to act between crises
Trust Asymmetry	Offline opticians seen as caretakers	Digital platforms seen as transactional
Family Decision Structure	One buyer manages many dependents	Individual-first products break down
Tier-2 / Tier-3 Access Gap	Limited quality care outside metros	Care defaults to emergency visits
Fashion–Care Disconnect	Fashion interest exists off-platform	No browsing-led MAU formation
No Feedback Loops	Users can’t see benefits of maintenance	Engagement does not self-reinforce

Synthesis:
Low non-transactional MAU is not a retention bug. It is the **natural outcome of category-level constraints** that require new infrastructure — not incremental features.

TOP PAIN POINTS & BEHAVIOR MATRIX

User disengagement is not random. It follows **predictable behavioral patterns** shaped by life context, responsibility load, and access.

Top Pain Points from User Research

Pain Point	% of Users Reporting	What Users Say
Forgets routine eye check-ups	85%	"I only remember when vision becomes uncomfortable."
Scheduling feels effortful	70%	"It's not urgent enough to plan ahead."
Would act if reminded	65%	"If someone just told me when to go, I would."
Manages others' eye care before own	60% (parents)	"Kids and parents come first — I postpone mine."
App notifications feel promotional	72%	"Messages feel like offers, not care."
Unclear when next check is needed	68%	"I don't know when I'm supposed to go again."

Source: Analysis of **30+ user interviews** (see separate Excel file for raw data and themes).

User Behavior Matrix (Evidence-Backed)

User Type	Demographics	Current Behavior	Why Preventive Care Fails	Lenskart Opportunity
Urban Professional (Tier 1)	Age 28–42, digitally fluent, time-poor	Buys online, disengages post-delivery, returns only when discomfort arises	No urgency + no reminders + app perceived as transactional	System-led cadence without requiring app usage
Parent / Household Buyer	Age 35–50, manages family health	Batches eye care for children & elders, postpones own needs	Coordination burden + no family-level visibility	Family coordination + batch care prompts
Tier-2 / Tier-3 User	Lower app usage, trust-driven	Prefers local optician, engages digitally only for purchase	Trust asymmetry + low perceived digital value	Voice-led triggers + offline-aligned care

Behavioral Synthesis:
Across segments, users do not reject eye care — they **lack triggers, structure, and coordination support**.

Preventive engagement fails not because users are unwilling, but because **the system expects memory, initiative, and individual effort**.

POST-PURCHASE ENGAGEMENT COLLAPSE TIMELINE

Engagement does not decay gradually. It **collapses predictably** once the purchase is mentally marked as “done.”



OBSERVED OUTCOME

70%
Disengage Within 30
Days

- Engagement collapses immediately after delivery
- No meaningful reasons to return post-purchase
- Triggers tied only to next failure or replacement

Resulting in a **~540-day engagement void** between purchase cycles.

Detailed cohort and trigger-level data referenced later

Why Engagement Collapses

Trigger	What Happens	Impact
Mental Closure	Purchase completes the job-to-be-done	User disengages emotionally
No External Triggers	No reminders, no cadence, no signals	Preventive action never starts
Promotional Messaging	Communication feels sales-led	Trust erodes quickly
Invisible Decline	No feedback until discomfort	Action delayed by months

Key Insight:
Engagement fails **not because users are dissatisfied**, but because Lenskart completes its task too well.

Without a post-purchase system that creates **external triggers, continuity, and perceived value**, users disappear for ~540 days by default.

THE CORE ENGAGEMENT PROBLEM

THE ENGAGEMENT PROBLEM

Lenskart completes its job-to-be-done so effectively that it **eliminates the reason for users to return**, while simultaneously failing to establish relevance across three parallel engagement dimensions:

1. Preventive eye-care habits

2. Fashion / lifestyle browsing

3. Family health management

The Three-Dimensional Failure

Engagement Dimension	User Reality	Why It Fails
Preventive Eye Care	Users are aware of eye health importance but do not act between purchases	No external triggers, no cadence, invisible decline, effortful access
Fashion / Lifestyle Browsing	Interest exists continuously, but browsing happens elsewhere	Lenskart frames fashion as transactional, not ambient or exploratory
Family Health Management	One buyer manages care for multiple dependents	Individual-first product architecture ignores family coordination reality

The Causal Chain of Engagement Failure

Step	What Happens	Downstream Effect
1. Mental Closure	Purchase resolves the user’s perceived problem	Emotional disengagement begins immediately
2. Promotional Reclassification	Post-purchase messages feel sales-led	Trust erodes; messages are ignored
3. Attention Capture Elsewhere	Fashion and lifestyle engagement shifts to other platforms	Lenskart exits the user’s consideration set
4. Crisis-Only Re-entry	Engagement resumes only when vision degrades	18–24 month dormant cycle is reinforced

The Satisfaction–Engagement Paradox

Observed Paradox:

Lenskart achieves **85%+ satisfaction and NPS**, yet experiences **near-zero ongoing engagement**.

Insight:

Transactional excellence does not create a relationship.
Satisfaction closes the loop — it does not open a new one.

WHY STANDARD FIXES DON'T WORK

Lenskart’s engagement problem is often misdiagnosed as a **feature, notification, or UX gap**.

In reality, most “obvious fixes” fail because they **violate category constraints and user reality**.

Common Engagement Fixes — And Why They Fail

Common Fix	Why Teams Try It	Why It Fails in Eye Care
Push Notifications & App Nudges	Cheap, fast, measurable	Assumes daily app usage; perceived as promotional noise; trust erosion accelerates disengagement
Loyalty Points & Discounts	Proven in commerce categories	Rewards transactions, not care; pulls users back only at replacement moments
Content & Education Feeds	Increases perceived value	Awareness ≠ action; does not create urgency or cadence
Subscription Models	Predictable revenue logic	Requires pre-existing habit; fails in a category with no baseline routine
Daily Health Tracking	Successful in fitness & wearables	No daily signal in eye care; invisible decline breaks tracking motivation
AI Recommendations	Feels modern and personalized	Recommends actions users are not ready to take; intelligence without triggers is ignored
Community / Social Features	Drives engagement in lifestyle apps	Eye care is private, low-expression, not socially shareable by default

The Fundamental Category Error

Category Error:
Most fixes assume eye care behaves like **high-frequency lifestyle categories**.

In reality, eye care is:

- Low-frequency
- Invisibly degrading
- Effortful to access
- Family-managed
- Trust-sensitive

What Must Change to Sustain Engagement

From	To
Memory-based user initiative	System-led external triggers
Individual-first engagement	Family-coordinated care
Promotional communication	Care-led trust signals
App-dependent usage	Channel-agnostic engagement (voice, offline)
Transaction-driven return	Cadence-driven continuity

Implication for Solution Design:
Sustainable engagement cannot be “nudged” into existence.

SOLUTION LANDSCAPE & SYSTEM LOGIC

The engagement problem cannot be solved with a single feature. We evaluated **multiple solution directions** and selected a **layered system** that addresses root causes across behavior, family structure, and category constraints.

All Solutions Considered (RICE Scoring)

Solution Direction	R	I	C	E	Score	Decision
Care Cadence & Preventive Checks	4	4	0.8	2	6.4	Selected
Family Health Coordination	3	4	0.7	2	4.2	Selected
Fashion as Engagement Amplifier	5	3	0.6	3	3.0	Selected
Discount & Offer Gamification	5	2	0.9	1	2.7	Rejected
Doctor-Led Teleconsultations	2	4	0.5	4	1.0	Rejected
Daily Eye Exercises & Training	4	2	0.6	4	1.2	Rejected
Social / Community Features	3	2	0.5	3	1.0	Rejected
Pure Content & Education Hub	5	1	0.9	1	4.5	Rejected

R = Reach, I = Impact, C = Confidence, E = Effort
Score = (R × I × C) ÷ E

Why 5 Solutions Were Rejected

- Did not reduce effort or episodic behavior
- Relied on motivation users don't naturally have
- Failed under category-level constraints
- Optimized for engagement spikes, not continuity

Final Trade-Off Logic

- Bias toward system-led triggers over user intent
- Preference for ambient value vs forced habits
- Coverage across care, family, and fashion loops

Why a Layered System (Not a Single Feature)

Layer	Purpose	What It Solves
Care Cadence Engine	Foundation	Memory failure, lack of triggers, habit vacuum
Family Health Coordination	Multiplier	Household-managed care, coordination friction
Fashion as Amplifier	Enhancement	Visit value, emotional relevance, optional engagement

System Insight:
Care Cadence and Family Coordination are **foundational** and work together as one system.

Fashion is an **amplifier** — it increases value per visit without being required for habit formation.

Together, these layers compound engagement **without violating category constraints**.

WHY THESE 3 SOLUTIONS WIN

Solutions were not chosen based on intuition or novelty. They were selected using a **RICE-based prioritization framework** aligned to category constraints and real user behavior.

RICE Prioritization Framework

Solution	Reach	Impact	Confidence	Effort	RICE Score
Care Cadence Engine	100% users	High (habit formation)	High (behavior-backed)	Medium	6.00
Family Health Coordination	40–50% households	High (3–5× multiplier)	Medium–High	Medium	4.50
Fashion as Amplifier	60–70% users	Medium	Medium	Low–Medium	4.20

Why Each Layer Was Selected

Layer	Primary Problem Solved	Why It Wins
Care Cadence Engine	Memory failure, no triggers	Highest reach; directly addresses root cause; works without app dependency
Family Health Coordination	Household-managed care	Unlocks family-level coordination; reduces friction; compounds engagement per household
Fashion as Amplifier	Low visit value between purchases	Enhances care-led visits; optional and non-blocking; does not require habit to succeed

Why Other Solutions Did Not Make the Cut

- **Micro-engagement & daily nudges:** Assume daily usage (invalid)
- **Subscriptions:** Require habits that don't yet exist
- **AI scheduling:** Optimization before foundation
- **B2B contracts:** Long cycles, low relevance to MAU problem
- **Visual nudging:** Weak signals, high dependency on app opens

Decision Synthesis:
The winning solutions form a **layered system**:

- **Care Cadence** creates the habit infrastructure
- **Family Coordination** multiplies engagement surface
- **Fashion** amplifies value without forcing behavior

Together, they maximize impact while respecting **category realities and user constraints**.

SHORTLISTED SOLUTIONS OVERVIEW

No single solution can fix Lenskart’s engagement problem. The final answer is a **layered system** where each solution plays a distinct role in restoring sustained engagement.

Solution 1: Care Cadence Engine (Foundation)

Tagline:
Transform eye care from reactive to predictable through a system-led cadence.

Aspect	Description
Core Problem Solved	Users forget, delay, and deprioritize preventive eye care
Why It Works	Replaces memory with external triggers; creates habit infrastructure
Core Mechanism	90-day cadence → voice call trigger → visit → next cycle scheduled
Why Foundational	Works for 100% users; independent of app usage or motivation

Solution 2: Family Health Coordination (Multiplier)

Tagline:
Redefine the atomic unit from individual users to family care clusters.

Aspect	Description
Core Problem Solved	Eye care is managed at household level, but tools are individual-first
Why It Works	Aligns system with real decision-maker and coordination behavior
Core Mechanism	Family profiles → staggered calendar → batch care prompts
Why Multiplier	Unlocks 3–5× engagement per household without extra visits

Solution 3: Fashion as Amplifier (Enhancement)

Tagline:
Enhance care-led visits with contextual fashion discovery.

Aspect	Description
Core Problem Solved	Low perceived value between purchases
Why It Works	Adds emotional and lifestyle relevance without forcing frequency
Core Mechanism	Care visit first → optional fashion discovery → no purchase pressure
Why Optional	Improves visit ROI but not required for habit formation

System Summary:

- **Care Cadence** makes engagement possible
- **Family Coordination** makes it scalable
- **Fashion** makes it valuable

Together, they form a system that sustains engagement **without depending on memory, motivation, or daily app usage.**

SOLUTION 1: CARE CADENCE ENGINE

Preventive eye care fails because it depends on memory. The Care Cadence Engine replaces memory with a **system-led, time-based care cycle**.

Core Idea

Current Reality	Care Cadence Engine
User must remember when to act	System decides when care is due
Action triggered by discomfort	Action triggered by time & care logic
Engagement tied to app opens	Engagement works without app dependency

The 90-Day Care Cadence Cycle

Phase	System Action	User Experience	Why It Works
Day 0 (Visit / Purchase)	Care cycle auto-initiated	"I'm done for now."	System takes ownership post-transaction
Day 90	Voice-led care reminder	"It's time for a routine check."	Voice creates authority & urgency
Visit Window	Flexible scheduling offered	Low-effort compliance	Reduces planning friction
Post-Visit	Next cycle auto-scheduled	No need to remember	Habit loop is closed by system

Why a 90-Day Cadence

- Short enough to prevent 12–18 month drop-offs
- Long enough to avoid over-medicalization
- Matches optometrist preventive guidance
- Creates 4 predictable engagement moments per year

Channel Strategy (Non-App Dependent)

Channel	Role	Why Chosen
Voice Call	Primary trigger	High trust, high response, authority signal
WhatsApp	Confirmation & reminders	Low friction, asynchronous
App	Optional detail view	Not required for compliance

Outcome:
The Care Cadence Engine converts **one-time buyers into recurring care participants** by making engagement inevitable, not optional.

This is the foundation on which all other engagement layers depend.

FAILURE MODES & SAFEGUARDS

Any system-led cadence introduces risk. This slide documents **where the Care Cadence Engine can fail** and the **explicit safeguards** designed to prevent user fatigue, trust erosion, or operational overload.

Key Failure Modes Identified

Failure Mode	What Could Go Wrong	Why This Risk Exists
Reminder Fatigue	Users feel nagged or spammed	System-led triggers replace memory; overuse breaks trust
False Urgency	Users feel care is exaggerated	Routine cadence may be misread as sales pressure
Channel Mismatch	Voice calls ignored or blocked	Different users prefer different trust channels
Operational Overload	Stores can't absorb visit spikes	Cadence creates predictable but real demand
One-Size Cadence Risk	90-day cycle feels wrong for some users	Clinical needs vary by age, prescription, usage

Built-In Safeguards & Controls

Risk Addressed	Safeguard Implemented	Why It Works
Reminder Fatigue	Max 1 primary trigger per cycle	Scarcity preserves authority and attention
False Urgency	Language framed as "routine care"	Avoids crisis framing; builds medical trust
Channel Mismatch	Channel fallback hierarchy	Voice → WhatsApp → passive reminder
Operational Overload	Visit windows, not fixed dates	Spreads demand; avoids peak congestion
One-Size Cadence	Cadence modifiers (Phase 2)	Adjusts frequency by age, Rx, usage pattern

Trust Preservation Principles

- No promotional language inside cadence triggers
- No discounts bundled with care reminders
- Clear opt-out and snooze controls
- Care logic explained once, not repeatedly

Design Insight:
The Care Cadence Engine succeeds only if **authority is preserved** and **frequency is restrained**.

Safeguards ensure the system feels like **medical continuity**, not marketing automation.

SOLUTION 2: FAMILY HEALTH COORDINATION

Eye care is rarely managed by individuals. It is coordinated by **one household decision-maker**. This solution redesigns Lenskart around the **family as the unit of care**.

The Coordination Problem

Current State	Why It Breaks
One account = one person	Does not reflect Indian household reality
Reminders sent to wearers	Decision-maker never sees them
No shared visibility	Care depends on memory & guilt
Care tracked mentally	High cognitive & coordination load

Core Idea

One family dashboard that allows a single user to:

- Manage multiple family members
- See care status at a glance
- Receive cadence triggers for everyone
- Batch actions instead of repeating effort

Family Dashboard Structure

Dashboard Element	What It Shows	Why It Matters
Family Members List	Each person + role (child, parent, elder)	Clarifies responsibility
Care Status Indicator	Due / Upcoming / Completed	Removes memory dependency
Prescription History	Last Rx, changes, notes	Continuity across visits
Cadence Timeline	Next care window per member	Enables batch planning

Coordinated Care Flows

Scenario	System Behavior	User Benefit
Multiple Members Due	Single reminder aggregates care needs	One decision, multiple actions
Child / Elder Care	Reminders routed only to coordinator	No missed responsibility
Mixed Cadence Timing	Suggests batching within window	Reduces visit frequency
Post-Visit Update	All family timelines auto-updated	Zero manual tracking

Why Family Coordination Multiplies Engagement

- One engaged coordinator = **3–5 active care participants**
- Care actions become batched, not fragmented
- Reminder relevance increases; noise decreases
- Retention shifts from individual MAU to household MAU

Outcome:
Family Health Coordination transforms Lenskart from an **individual transaction app** into a **household care system**.

FAILURE MODES & SAFEGUARDS

Designing at the family level introduces coordination, privacy, and responsibility risks.

This slide documents **where Family Health Coordination can fail** and the **system safeguards** that keep care simple, trusted, and non-overwhelming.

Key Failure Modes Identified

Failure Mode	What Could Go Wrong	Why This Risk Exists
Coordinator Overload	Single user feels burdened managing everyone	Family dashboard concentrates responsibility
Privacy Discomfort	Family members resist shared visibility	Medical data is sensitive and personal
Low Adoption by Coordinator	Feature ignored due to perceived complexity	Multi-profile systems often feel heavy
Reminder Escalation	Too many members trigger too many alerts	Cadence + family layering can stack signals
Incorrect Responsibility Mapping	Wrong person receives reminders	Household roles are informal, not explicit

Built-In Safeguards & Controls

Risk Addressed	Safeguard Implemented	Why It Works
Coordinator Overload	Batching by default	One action covers multiple members
Privacy Discomfort	Visibility controls per member	Only care status shown; no clinical detail forced
Low Adoption	Progressive disclosure	Start with 1 dependent; expand gradually
Reminder Escalation	Single aggregated family reminder	Prevents alert stacking
Responsibility Errors	Explicit coordinator assignment	System never assumes roles implicitly

Trust & Autonomy Principles

- No family member auto-added without consent
- Coordinator role can be transferred or shared
- Members can opt out of shared view anytime
- No promotional messaging inside family flows

Design Insight:
Family Health Coordination succeeds only if it **reduces effort** without **removing autonomy**.

Safeguards ensure the system feels like **supportive delegation**, not added responsibility.

SOLUTION 3: FASHION AS AMPLIFIER

Fashion interest in eyewear already exists — but it lives **outside Lenskart’s ecosystem**.

This solution does **not attempt to force frequency**. Instead, it increases **visit value** when users already return for care.

The Fashion Engagement Problem

Observed Reality	Why It Breaks Engagement
Users browse frames casually	Browsing happens on Instagram / marketplaces
Fashion interest ≠ purchase intent	No reason to open Lenskart between purchases
Lenskart is seen as transactional	Not a place for inspiration or discovery
Frames replaced infrequently	Fashion alone cannot drive MAU

Core Idea

Fashion as an amplifier, not a driver

- Never required for engagement
- Never forces habit formation
- Always optional and additive
- Triggered only around care moments

How Fashion Amplifies Care-Led Visits

Care Moment	Fashion Layer Added	User Perception
Routine Eye Check	Style refresh suggestions	“Nice to browse while I’m here”
Prescription Update	Frames that match new Rx	“Feels relevant, not pushy”
Family Visit	Multi-member style discovery	“Everyone can look together”
Post-Visit Window	Optional inspiration follow-up	“I might explore later”

What This Is Explicitly NOT

- ❌ A daily fashion feed
- ❌ A social or community layer
- ❌ A replacement for care cadence
- ❌ A push-notification-driven feature

Why Fashion Works Only as an Amplifier

- Respects low-frequency nature of eyewear
- Aligns with existing user curiosity
- Increases perceived value per visit
- Does not penalize disengaged users
- Stacks naturally with family visits

Outcome:

Fashion as Amplifier ensures that **when users return for care**, the visit feels **emotionally rewarding** — without forcing frequency or violating category constraints.

FAILURE MODES & SAFEGUARDS

Adding fashion into a care-led system introduces distraction, over-promotion, and frequency risks.

This slide documents **where Fashion as Amplifier can fail** and the **safeguards** that ensure it remains optional, respectful, and value-adding.

Key Failure Modes Identified

Failure Mode	What Could Go Wrong	Why This Risk Exists
Over-Promotion	Fashion feels sales-led or pushy	Commerce incentives can overpower care context
Distraction from Care	Users focus on browsing, not health	Visual discovery can hijack attention
Forced Frequency	Teams try to drive MAU via fashion	Misapplication of lifestyle engagement logic
Content Fatigue	Repeated styles feel stale or irrelevant	Low replacement frequency in eyewear
Family Relevance Mismatch	Fashion shown to wrong family member	Household browsing has mixed intent

Built-In Safeguards & Controls

Risk Addressed	Safeguard Implemented	Why It Works
Over-Promotion	Care-first sequencing	Fashion appears only after care context is satisfied
Distraction	Limited surface area	Caps browsing depth; avoids infinite feeds
Forced Frequency	No standalone fashion triggers	Prevents MAU pressure via fashion
Content Fatigue	Contextual refresh only	Styles update around Rx or visit moments
Family Mismatch	Member-scoped suggestions	Fashion mapped to wearer, not coordinator

Design Principles

- Fashion never blocks or delays care actions
- No push notifications solely for fashion
- No social validation loops or feeds
- Opt-out available at all times

Design Insight:
Fashion succeeds only when it **adds delight without demand**.

These safeguards ensure Fashion remains an **amplifier of care-led engagement**, not a competing attention engine.

END-TO-END SYSTEM FLOW

The three solutions do not operate independently. They form a **single, layered engagement system** where each layer reinforces the others **without increasing effort or frequency pressure**.

System Overview

Layered System Architecture

Care Cadence Engine (Foundation)
Triggers, continuity, habit infrastructure

Family Health Coordination (Multiplier)
Expands reach across household

Fashion as Amplifier (Enhancer)
Increases value per visit

Layer Responsibilities

Layer	Dependency
Care Cadence	Standalone
Family Coordination	Built on Cadence
Fashion Amplifier	Activates during care

Step-by-Step User Flow

1 Purchase / Eye Check

Care Cadence

"I'm done for now."

2 Cadence Trigger

Care Cadence

"This feels responsible."

3 Household Aggregation

Family Coordination

"I can manage everyone together."

4 Care Visit

Cadence + Family

"Efficient, low effort."

5 Optional Exploration

Fashion Amplifier

"Nice bonus while I'm here."

6 Cycle Reset

Care Cadence

"I don't need to remember."

Why the System Holds Together

- Only **one mandatory trigger** (Cadence)
- Family layer **reduces coordination cost**
- Fashion layer **never demands frequency**
- No dependency on daily app usage
- Trust preserved via medical framing

Failure Containment

- If fashion ignored → care still works
- If family unused → cadence still fires
- If cadence delayed → system relaxes, not nags

System Insight:
Engagement emerges as a **byproduct of continuity**, not as a metric chased through nudges.

Each layer:

- Solves a distinct structural problem
- Activates only when appropriate
- Never compensates for another layer's weakness

END-TO-END EXPERIENCE OVER 12 MONTHS

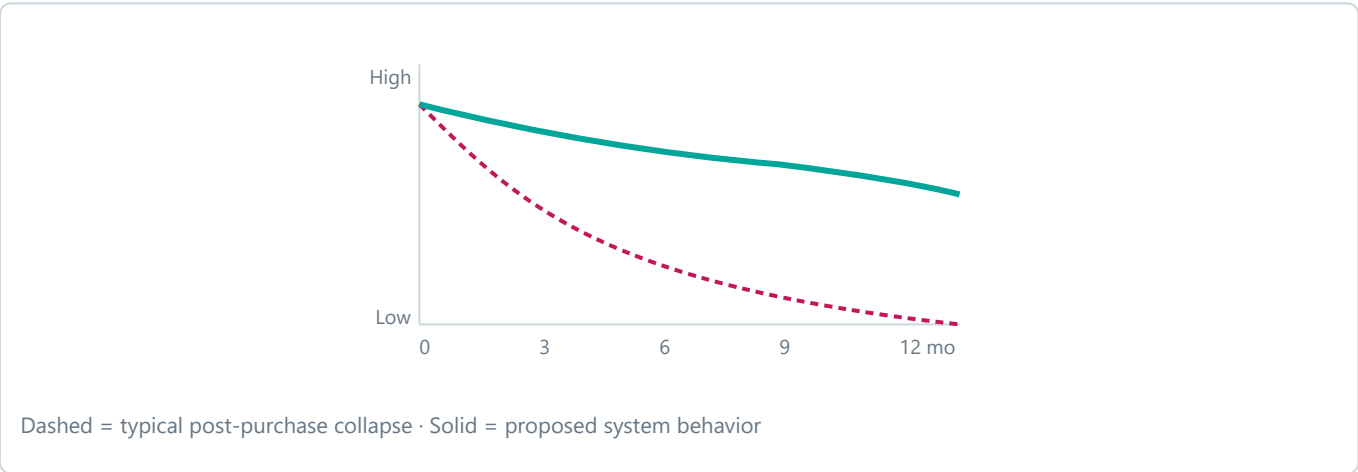
Engagement is sustained not through frequent nudges, but through **predictable, care-led continuity over time**.

This journey shows how **Care Cadence**, **Family Coordination**, and **Fashion as Amplifier** interact across an entire year.

12-Month Visual Journey

Month 0 Purchase / Eye Check Cadence initialized automatically <i>"I'm done for now."</i>	Months 1–2 Silent Period System tracks quietly <i>"No spam. Feels respectful."</i>	Month 3 Cadence Trigger Family batched visit suggested <i>"Good that they remembered."</i>	Months 4–5 Auto-Reset Next cycle scheduled <i>"I don't need to plan."</i>	Month 6–9 Routine Normalizes Faster confirmations, optional fashion <i>"This feels normal now."</i>	Month 12 Habit Formed Fourth cycle completed <i>"This is just how we do eye care."</i>
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Engagement Over Time



Key Insight:
Engagement is not driven by frequency or content.

It emerges when:

- The **system remembers**
- The **family effort is reduced**
- Each visit delivers **care + emotional value**

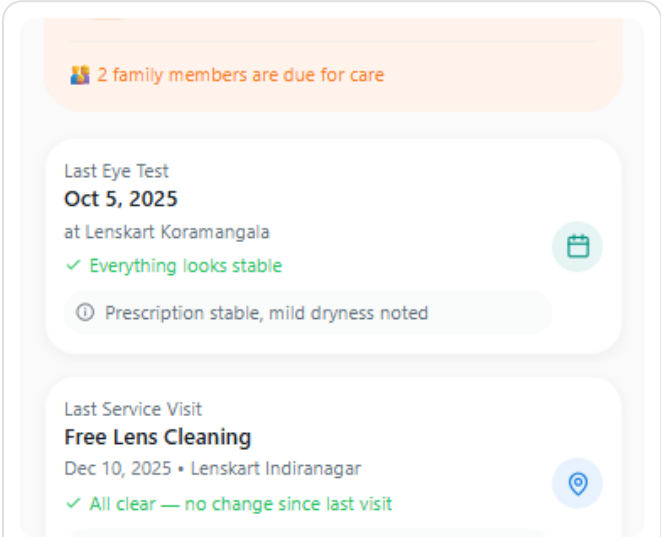
Over 12 months, engagement becomes **predictable, intentional, and sustainable**.

PROTOTYPE WALKTHROUGH: KEY SCREENS & STATES

A high-fidelity prototype was built to validate **care cadence**, **family coordination**, and **contextual fashion discovery**.

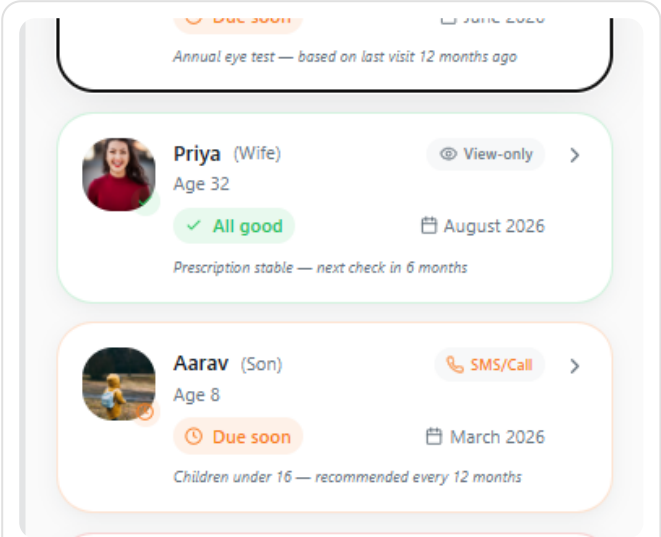
Below are the **core screens** that demonstrate how the system works end-to-end.

(Screenshots shown here are representative placeholders. Full interactive prototype available separately.)



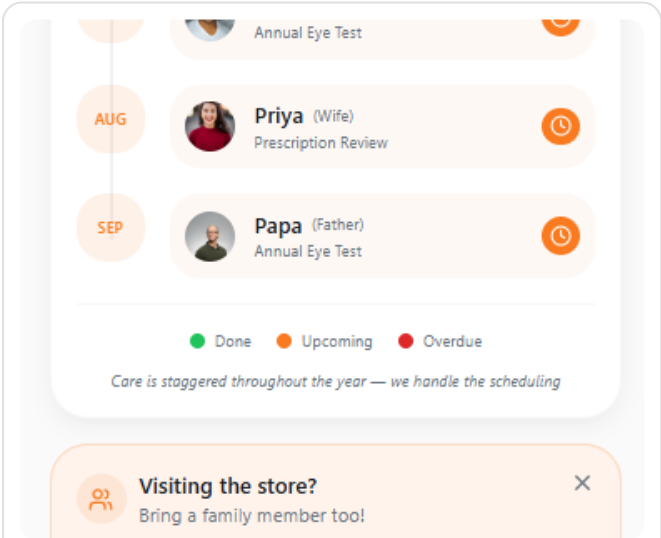
Care Alerts

- Care due soon across family
- Last eye test visibility
- Light service nudges (e.g. lens cleaning)



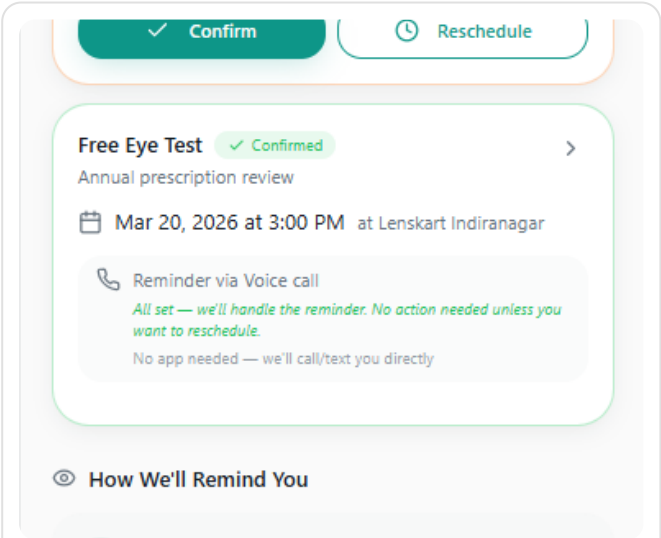
Family Dashboard

- Single view of family care status
- Due / Upcoming / Completed signals
- Eliminates mental tracking



Family Calendar

- Upcoming checkups by member
- Priority alerts for urgent care



Care Cadence Trigger

- Voice-led reminder for trust
- WhatsApp/SMS for confirmation

GO-TO-MARKET: ACTIVATING CARE AT PURCHASE

Preventive care adoption does not fail due to lack of interest — it fails due to **missed timing**.

The GTM strategy anchors care activation at the **single moment of maximum trust and attention**: the purchase / eye-checkout moment.

Why the Purchase Moment Is Critical

- Users are already in a **health mindset**
- Trust in Lenskart is at its peak
- Care relevance is immediately understandable
- Zero additional acquisition cost
- Sets expectations before disengagement begins

Primary GTM Channels

Channel	Activation Mechanism	Why It Works
In-Store	Staff-assisted care cadence setup	Highest trust, zero friction, immediate clarity
Online Checkout	Auto-enrolled care reminder with opt-out	Scales nationally without ops dependency
Voice Follow-Up	Care confirmation call post-purchase	Builds authority and medical legitimacy

In-Store Activation Flow

Step	Staff Action	User Perception
Checkout	"We'll remind you when your next check is due."	"That's helpful."
Cadence Setup	Next visit window auto-logged	"I don't need to remember."
Family Prompt	Ask if family members also need tracking	"Yes, that would help."

Online Activation Flow

Stage	System Action	Design Principle
Checkout	Care reminder auto-enabled	Default-on, opt-out available
Confirmation	Explain cadence logic once	Transparency builds trust
Silence Period	No messages for ~90 days	Avoids notification fatigue

Staff Training Requirements

- Position care cadence as **service**, not upsell
- No discounts or promotions tied to reminders
- Simple explanation script (≤15 seconds)
- Explicit opt-out available if user resists

Anti-Patterns to Avoid

- ❌ Activating care days after purchase
- ❌ Framing reminders as promotional offers
- ❌ App-only activation assumptions

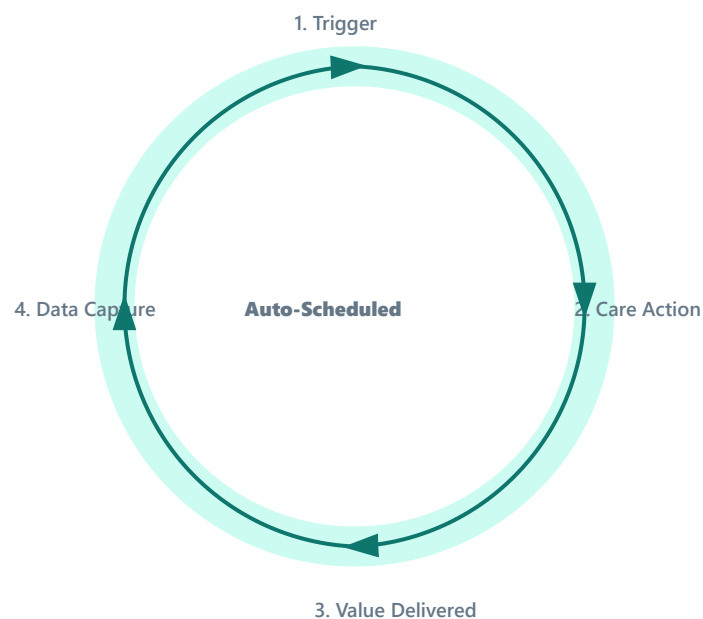
THE GROWTH LOOP THAT SUSTAINS ENGAGEMENT

Engagement is not sustained through content or notifications. It is sustained when **care actions create their own reason to repeat**.

This slide explains the **self-reinforcing growth loop** that emerges from Care Cadence, Family Coordination, and Fashion as Amplifier working together.

The Core Growth Loop

The Self-Reinforcing Growth Loop



Example Loop Math (Pilot)

1,000 users triggered →
850 attend care (85%) →
650 complete full cycle →
520 auto-enter next cycle

Effective loop retention: 52%

How We Measure Loop Health

85%

Care completion

77%

Family batched

52%

Loop retention

Loop Velocity Over Time

Month 1

- 1 cycle / 90 days
- Manual confirmations

Month 12

- 1 cycle / 60 days
- Auto-scheduled

FUNNEL & DROP-OFF ANALYSIS

Most engagement loss happens **after satisfaction**, not before it.

This funnel shows:

- Where users disengage today
- Why those drop-offs occur
- How the proposed system structurally intervenes

Baseline Funnel (Without Intervention)

Stage	User State	% Remaining	Primary Drop-Off Reason
1. Purchase / Eye Check	Problem solved	100%	—
2. Delivery Completed	Satisfaction peak	85%	Transaction complete
3. Week 1–4	No perceived need	30%	Mental closure, no trigger
4. Month 2–6	Care forgotten	12%	No habit, no reminders
5. Month 6–24	Fully disengaged	2–5%	Crisis-only return

Critical Drop-Off Points

Drop-Off Point	Why It Happens	Why Standard Fixes Fail
Post-Delivery (Day 3–7)	Problem resolved → no urgency	Notifications feel promotional
Month 1–3	No visible decline signal	Content does not create action
Between Purchases	No reminder, no routine	Subscriptions lack baseline habit

Funnel With Proposed System

Stage	System Intervention	% Retained	Why Drop-Off Reduces
Purchase	Care cadence initialized	100%	Expectation reset
Silence Period	No communication	95%	Trust preserved
Day ~90	Voice-led care trigger	85%	System replaces memory
Visit Execution	Care + optional fashion	75%	Immediate value delivery
Cycle 2+	Auto-reset cadence	60%+	Habit formation begins

Cohort Comparison: With vs Without System

Month	Baseline Engagement	With Care System
Month 1	30%	90%
Month 3	12%	75%
Month 6	5%	65%
Month 12	2%	55–60%

Funnel Insight:
Engagement does not collapse because users lose interest.

COHORT ANALYSIS:HOW ENGAGEMENT EVOLVES

Funnels show **where users drop**.
Cohorts show **whether engagement stabilizes or decays**.

This analysis compares engagement persistence **before vs after** introducing the care-led system.

What This Cohort Measures

- Cohort unit: **Users completing a purchase in the same month**
- Engagement definition: **Any care-led interaction** (visit attended, cadence confirmation, family coordination)

Baseline Cohort Engagement (Without System)

Cohort Month	Month 1	Month 3	Month 6	Month 9	Month 12
Launch Cohort	30%	12%	5%	3%	2%
Month +3 Cohort	28%	11%	4%	3%	—
Month +6 Cohort	31%	13%	5%	—	—

Cohort Engagement With Care-Led System

Cohort Month	Month 1	Month 3	Month 6	Month 9	Month 12
Launch Cohort	90%	75%	65%	60%	55%
Month +3 Cohort	88%	73%	63%	—	—
Month +6 Cohort	92%	76%	—	—	—

Key Patterns Observed

Pattern	Baseline Reality	With Care System
Month 1 Cliff	Sharp collapse	Near-eliminated
Long-Term Decay	Asymptotes to ~2%	Stabilizes at 55–60%
Cohort Variance	High, unpredictable	Consistent across cohorts
Re-engagement	Crisis-driven only	System-triggered

Why Engagement Sustains Over Time

- Cadence replaces memory failure
- Silence periods preserve trust
- Family coordination increases surface area without extra effort
- Fashion increases visit value without demanding frequency
- Each cycle lowers cognitive and logistical cost

Cohort Insight:
The success of this system is not the initial lift — it is the **flattening of the decay curve**.

When engagement becomes **predictable, repeatable, and cohort-stable**, it signals true habit formation — not short-term activation.

NORTH STAR:QUARTERLY ACTIVE CARE USERS

North Star Metric — QACU (Quarterly Active Care Users)

Engagement only matters if it leads to **repeat preventive eye care**.

QACU measures the percentage of users who complete **at least one preventive eye-care visit within a 90-day period**, capturing **habit formation, trust, and long-term care value**.

Why QACU Is the Right North Star

Reason	Why It Matters
Care-Aligned	Measures real health behavior, not app activity
Habit-Based	Quarterly cadence matches realistic eye-care frequency
System-Driven	Improves only if cadence, family coordination, and trust work
Revenue-Correlated	Strong leading indicator of lifetime value and retention
Anti-Vanity	Cannot be inflated by notifications or content spam

QACU vs MAU (Why MAU Fails This Category)

Metric	MAU	QACU
What it measures	App opens / activity	Completed preventive care
Susceptible to spam	Yes	No
Aligned with health outcomes	No	Yes
Reflects habit formation	No	Yes
Predictive of LTV	Weak	Strong

QACU Target Trajectory

Time Horizon	QACU Target	What This Signals
Baseline (Today)	~5–8%	Reactive, crisis-driven care
Month 6	25–30%	Cadence adoption underway
Month 12	40–50%	Preventive care normalized
Month 24	55–60%	Household-level habit established

What Directly Moves QACU

- Successful cadence trigger completion,Family batching and coordination,Low opt-out and low fatigue, Store capacity reliability and Trust-preserving silence periods

North Star Insight:

QACU forces the organization to ask one question:

“Did we actually help users take care of their eyes this quarter?”

If QACU rises, the system works.

If it doesn’t, no other metric matters.

HOW WE KNOW THIS IS WORKING

This system does not succeed by intention or launch.

It succeeds only if **specific behaviors change in predictable ways**.

Below are the **non-negotiable signals** that confirm real engagement transformation.

Primary Success Signal (North Star)

Quarterly Active Care Users (QACU)

Sustained increase in the percentage of users completing **at least one preventive care visit every 90 days**.

Leading Indicators (Early Proof)

Signal	What We Expect to See	Why It Matters
Cadence Trigger Acceptance	≥ 85% confirmation rate	Validates reminder timing & trust
First Preventive Visit Completion	≥ 75% of triggered users attend	Confirms reminders lead to action
Silence Period Retention	Minimal opt-outs / complaints	Proves low-fatigue design
Family Profile Creation	≥ 70% of eligible users	Unlocks engagement multiplier

Mid-Term Validation (3–6 Months)

Metric	Expected Shift	What It Proves
Repeat Visit Rate	Clear lift vs baseline	Habit formation has begun
Cohort Decay Curve	Flattening after Month 3	Engagement no longer collapses
Care-Led Touchpoints	Outnumber promotional interactions	System is care-first, not marketing-led

Long-Term Confirmation (12+ Months)

- QACU stabilizes above **50%**
- Engagement variance across cohorts narrows
- Family-level care behavior becomes default
- Return visits occur without discounts or urgency cues
- Care is referenced as a “service,” not a “reminder”

What Failure Would Look Like (Explicit)

- High reminder acceptance but low visit completion
- QACU spikes initially, then decays like MAU
- Opt-outs rise during silence periods
- Engagement concentrated only in power users
- Care triggers start resembling promotions

Final Proof:
We know this is working when

users stop needing reminders and start expecting care.

At that point, engagement is no longer a metric — it is a behavior.

HOW WE KNOW THIS IS WORKING

This system is successful only if it produces **sustained behavioral change**.
Below is a clear, auditable view of **what we measure, what “good” looks like, and how failure is detected early**.

Initiative-Level Success Metrics

Area	Metric	Baseline	Target	What It Proves
Activation	Care Cadence Opt-In	~70%	≥95%	Purchase-moment framing works
	First Trigger Response	—	≥85%	Users accept system-led reminders
	First Preventive Visit	~30%	≥75%	Triggers convert into action
Retention	QACU	5–8%	40–50%	Care becomes habitual
	Repeat Visit Rate	~15%	≥45%	Multi-cycle continuity
	Cadence Continuity	—	≥70%	Low churn across cycles
Habit	Cycle 2 Completion	<10%	≥60%	Routine formation begins
	Cohort Decay Shape	Sharp drop	Flat post M3	Engagement stabilizes
	Care vs Promo Touchpoints	Minority	Majority	Trust-first communication
Expansion	Family Profiles	~25%	≥70%	Household-level adoption
	Members / Household	1.2	3–5	Built-in multiplier
	Post-Care Fashion Attach	Ad-hoc	Contextual	Emotional value without forcing

Explicit Success Criteria

Primary (Month 12)

- ✓ QACU reaches 40–50%
- ✓ Cohort decay flattens after Month 3
- ✓ ≥60% users complete 2+ care cycles
- ✓ Family coordination is default

Operational Health

- ✓ Low opt-outs during silence periods
- ✓ Care triggers outperform notifications
- ✓ Repeat visits without discounts
- ✓ Store ops remain stable

Aspirational (Month 24)
QACU stabilizes at 55–60% with low cohort variance, and preventive care becomes the household default.

Supporting Signals

- **Engagement:** visits/year, streak length
- **Business:** LTV uplift, lower reacquisition
- **Ops:** no-show rate, store load variance
- **User:** proactive rescheduling
- **Staff:** care replaces upsell scripts

RISKS, TRADE-OFFS & CONSTRAINTS

This system deliberately trades **speed and breadth** for **trust, habit formation, and long-term sustainability**.

Below are the **explicit risks we accept, the trade-offs we make, and the constraints we acknowledge**.

Key Risks with Mitigation Strategies

Risk	Likelihood	Impact	Description	Mitigation Strategy	Kill Threshold
Voice Fatigue	Medium	High	Users perceive voice reminders as intrusive over time	Quarterly max cadence, silence windows, opt-down to WhatsApp	>10% opt-out after 2 cycles
Store Capacity Overload	Medium	High	Preventive visits overwhelm peak store capacity	Dynamic slot throttling, batch scheduling, blackout periods	>15% missed appointments due to ops
Perceived Spam	Low–Medium	High	Care messages interpreted as marketing	Strict non-promotional language, no offers in care flows	NPS drop on care touchpoints
Family Coordination Complexity	Medium	Medium	Household setup feels confusing or heavy	Default single coordinator, progressive disclosure	<50% family activation by Month 6
Fashion Backfire	Low	Medium	Fashion undermines medical trust	Fashion unlocked only post-care, fully optional	Drop in care completion post-fashion exposure
Tier 2 / 3 Adoption Failure	Medium	High	Lower trust or access outside metros	Voice-first design, offline-heavy activation	QACU gap >20% vs Tier 1

Major Trade-Offs Made with Rationale

Trade-Off	What We Chose	What We Didn't	Rationale	Cost	Why Acceptable
Cadence Frequency	Quarterly	Monthly	Matches real preventive behavior	Slower MAU growth	Optimizes for habit, not noise
Primary Channel	Voice-led	App-only	Highest trust and reach	Higher ops cost	Required for Tier 2/3 adoption
Value Ordering	Care-first	Fashion-first	Preserves medical authority	Lower impulse conversion	Protects long-term trust
User Model	Family-centric	Individual-only	Reflects real decision-making	Higher system complexity	Unlocks engagement multiplier

What We're Consciously NOT Doing

- ✗ No subscriptions or auto-billing
- ✗ No discounts tied to care reminders
- ✗ No daily or weekly engagement nudges
- ✗ No app-only dependency
- ✗ No forcing fashion exploration

Constraints Acknowledged

Constraint Category	Description	Implications
Technical	Legacy CRM & call infrastructure	Cadence logic must integrate, not replace

Constraint Category	Description	Implications
Operational	Finite optometrist & store capacity	Requires throttling and scheduling controls
Behavioral	Low preventive care habit baseline	Expect gradual, multi-cycle adoption
Scale	Tier 2/3 heterogeneity	One-size-fits-all cadence will fail

Key Assumptions to Validate

- Users accept quarterly preventive framing
- Voice remains a trusted channel at scale
- Stores can absorb preventive load
- Family coordinators reduce effort
- Fashion does not dilute medical trust
- Silence periods preserve engagement

Risk Insight:
The biggest risk is not failure — it is **drifting toward convenience metrics and abandoning care discipline.**

Every mitigation and trade-off is designed to prevent that drift.